

Public Health Emergency Operations Center

"COUSP DRC"

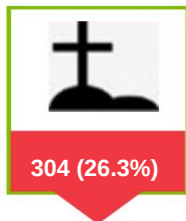
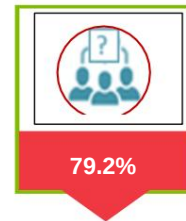
MVE-17 Incident Management System

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Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°041/MVB_24/06/2026

Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (34)	- Ituri (22/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia and Fataki. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	June 24, 2026
Publication date	June 25, 2026

Total confirmed cases
for the 3 provincesCumulative deaths
among confirmed cases
and case fatality ratePatients in
isolation -
hospitalizationCumulative number
of recoveriesSuspected cases of
the dayContact tracing rates
for the 3 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- **Thirty-seven (37) new confirmed cases, including 5 deaths**, were reported as of June 23, 2026, in the provinces of **Ituri (34 cases, including 4 deaths) and North Kivu (3 cases, including 1 death)**. These confirmed cases are distributed across the health zones of **Rwampara (12), Bunia (10), Nyankunde (5), Mongbwalu (4), Bambu (2), and Tchomia (1)** in Ituri province, and **Butembo (2) and Katwa (1)** in North Kivu province. • In addition, **8 deaths were recorded** among Ebola patients in Ebola Treatment Centers (ETCs) in the province from Ituri (4 in Bunia, 2 in Mongbwalu, 1 in Nyankunde and 1 in Rwampara).
- **Launch of the 90-day cross-border response plan** between the DRC and Uganda: the establishment and strengthening of joint laboratory capacities as well as clinical care facilities are the priorities of this plan.
- **Sixteen (16) patients** with Ebola virus disease in **Ituri** have been declared cured after obtaining negative control tests.
- **A confirmed case of Ebola virus disease has been reported in France**. It involves an intensive care physician who participated in the Ebola response in Bunia on behalf of the NGO Alima; he developed symptoms during a flight to France, where he currently resides.
- An expert from the National Institute of Public Health (INSP-DRC), deployed in Ituri, was evacuated to Kinshasa for better care after experiencing a spike in blood pressure in Bunia. Sadly, he died in Kinshasa on June 24, 2026. However, two samples were taken and the results were negative for Ebola virus disease (EVD).

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

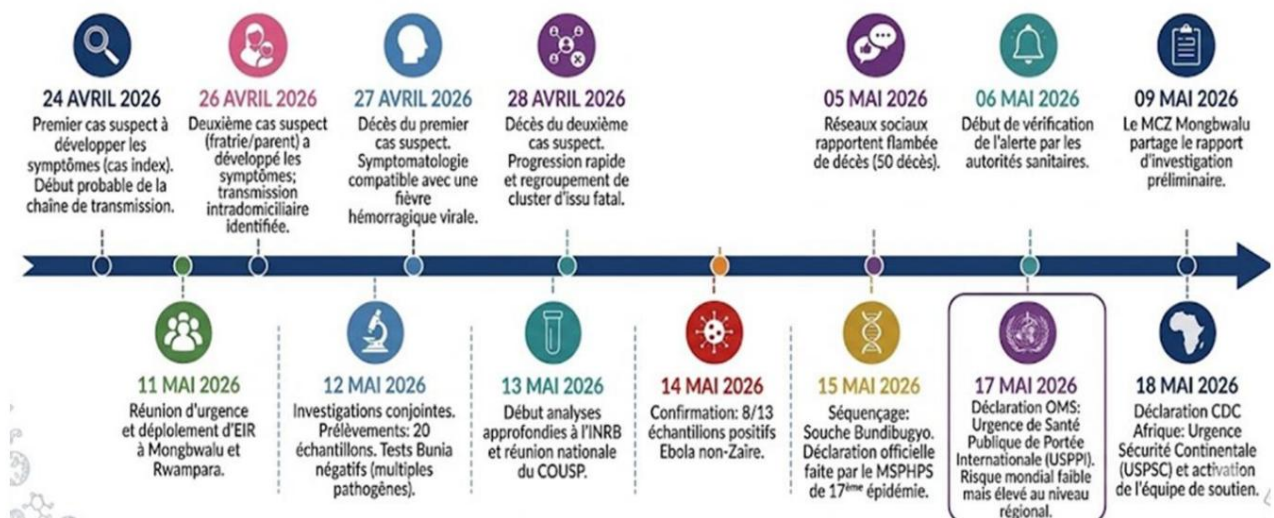


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 24, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of June 24, 2026.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New case confirmed (24h)
Ituri	1054	250*	23.7%	22 out of 36 (61.1%)	34
North Kivu	98	53	54.1%	11 out of 34 (32.3%)	3
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	1155	304	26.3%	34 out of 104 (32.7%)	37

*For the province of Ituri, **8 deaths were recorded** among patients with Ebola virus disease in Ebola treatment centers (4 in Bunia, 2 in Mongbwalu, 1 in Nyankunde and 1 in Rwampara), bringing the cumulative number of deaths among confirmed cases to 250.

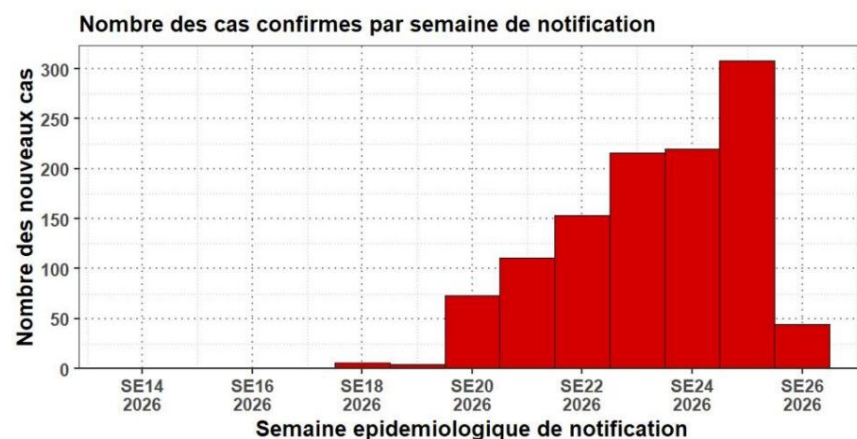
No new health zones were affected as of June 24, 2026. The number of affected health zones remains at 34/104 (section 3.1).

Since May 26, 2026, South Kivu province has not recorded any confirmed cases, suggesting an absence of recent transmission.

3.2 Distribution by health zone (Cumulative total as of 24/06/2026)

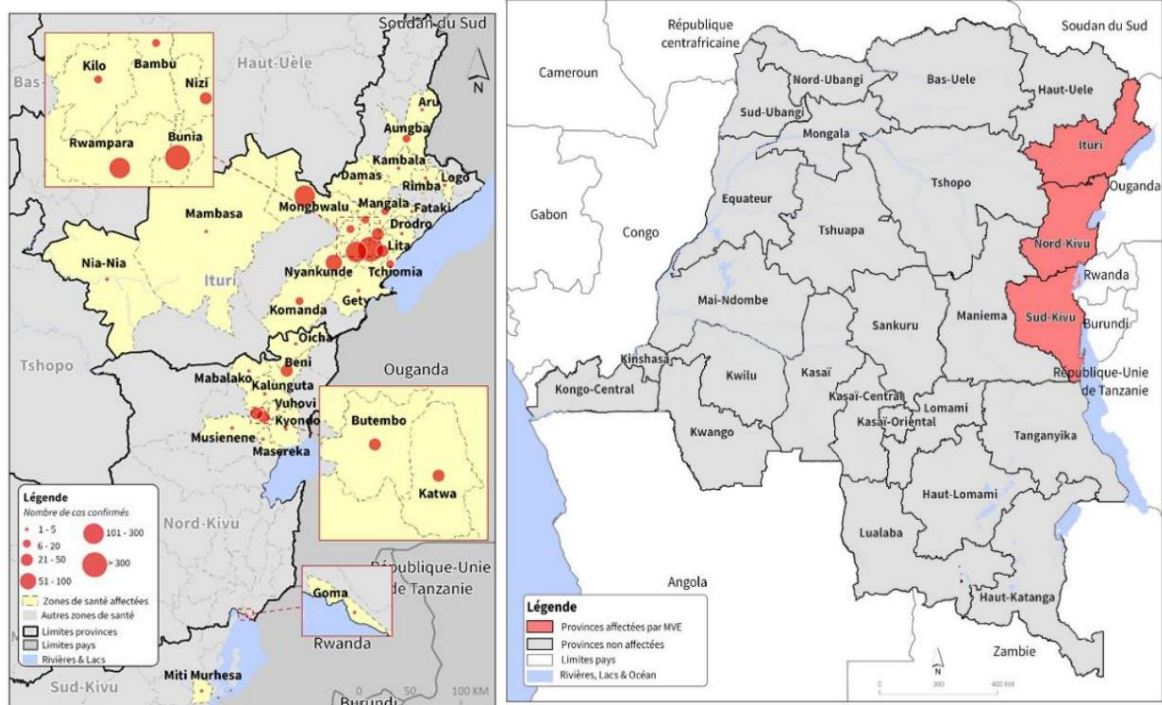
- **Ituri Province (1,054 cases):** Bunia (314), Mongbwali (245), Rwampara (237), Nyankunde (91), Nizi (30), Lita (26), Mangala (17), Bambu (14), Komanda (13), Kilo (9), Tchomia (9), Aungba (6), Logo (5), Nia-Nia (5), Damascus (4), Rimba (3), Aru (3), Mambasa (2), Drodro (1), Gety (1), Kambala (1), and Fataki (1). Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.
- **North Kivu Province (98 cases):** Katwa (33), Butembo (31), Beni (20), Oicha (3), Kyondo (3), Kalunguta (2), Musienene (2), Goma (1), Masereka (1), Vuhovi (1) and Mabalako (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Time-bound analysis: We observe an increasing number of confirmed cases from one week to the next, reflecting ongoing community transmission of the disease, although during epidemiological weeks 23 and 24, the number of cases appeared to be stable. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capacities) contributed to early detection, confirming continued and increased community transmission in week 25. Since week 26/2026 is not yet complete, we cannot compare it to previous weeks.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 1135.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 24, 2026

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (June 24, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Lethality (CFR, %)
ITURI	1054	250	23.7%
Bunia	314	60	19.1%
Rwampara	237	40	16.9%
Mongbwalu	245	105	42.9%
Nyankunde	91	10	11.0%
Nizi	30	6	20.0%
Lita	26	6	23.1%
Komanda	13	3	23.1%
Bamboo	14	2	14.3%
Kilo			11.1%
Mangala	9 17	1	47.1%
Tchomia	9	8	0.0%
Damascus	4	0	0.0%
Aungba	6	0	33.3%
Rimba	3	2	0.0%
Aru	3		33.3%
Mambasa	2		50.0%
Logo	5	0	0.0%
Drodro	1	1 1 0 1	100.0%
Gety	1	0	0.0%
Kambala			100.0%
Nia-Nia	1	1	60.0%
Fataki	5 1	3	0.0%
Other areas not yet identified	17	0	0.0%
NORTH KIVU	98	0	54.1%
Butembo	31	53	41.9%
Katwa	33	13	60.6%
Blessed	20	20	60.0%
Oicha	3		66.7%
Kalunguta	2		50.0%
Kyondo	3	12 2 1 2	66.7%
Goma	1	0	0.0%
Masereka	1	0	0.0%
Vuhovi	1		100.0%
Mabalako	1	1	0.0%
Musienene	2	0	100.0%
SOUTH KIVU	3	2 1	33.3%
Miti-Murhesa 1			33.3%
TOTAL 304	3 1155		26.3%

The epidemiological situation of the 17th Ebola Virus Disease (EVD) outbreak in the Republic

The Democratic Republic of Congo continues its spread, **with 37 new confirmed cases, including 5 deaths** .

recorded in the last 24 hours, bringing the total to **1,155 confirmed cases** and **304 deaths**, expressing an overall case fatality rate of **26.3%**.

Provincial dynamics and case concentration

Approximately **91.3%** of cases (n=1,054) and **82.2%** of deaths (n=250), representing a case fatality rate of **23.7%**, are concentrated in **Ituri** province, making it the absolute epicenter of the epidemic. The health zones of Bunia (314 cases), Mongbwalu (245 cases), and Rwampara (237 cases) account for 75.5% of all reported cases in this province, and are therefore the most affected. Furthermore, the health zones of Bunia and Rwampara have moderate case fatality rates (19.1% and 16.9% respectively), while the **Mongbwalu** zone shows marked severity, with a case fatality rate of **42.9%** (105 deaths out of 245 cases), suggesting persistent challenges in early intervention or access to care. It should be noted that 17 additional cases (with no associated deaths) cases have been reported, while the health zones of origin remain to be identified, which could indicate a geographical spread requiring further investigation.

Severity of the epidemic in North Kivu

A total of **98 confirmed cases** and **53 deaths** have been reported in this province, representing a case fatality rate of 54.1%. This last one presents the most worrying profile in terms of severity, higher than the national average. This high mortality rate is driven by the health zones of **Katwa** (60.6%), **Beni** (60.0%), and **Oicha** (66.7%). The Katwa health zone had the highest number of reported cases (n=33), followed by the Butembo health zone (n=31) with a lower case fatality rate (41.9%). A case fatality rate of 100% was exceptionally observed in the Vuhovi zone (with a small sample size), highlighting the virus's danger in this region, and the need for an immediate clinical response.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Launch of the 90-day cross-border response plan between the DRC and Uganda: the establishment and strengthening of joint laboratory capacities as well as clinical care facilities are the priorities of this plan, starting with the strategic area of Aru.
- Supervision of field teams and the various patient care structures.
- Holding the coordination meeting and supporting the pillars of the response.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 24, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Report (D-1)		10	79	5	94
New Alerts		204	586	19	809
Total alerts for the day		214	665	24	903
Alerts investigated		214	507	23	744
Investigation rate		100.0%	76.2%	95.8%	82.4%
Alerts confirmed	Alive	63	45	6	114
	Deceased	34	6	0	40
Total suspected cases for today		97	51	6	154

During the course of June 24, 2026, **903 alerts** were raised across the three provinces, **744 (82.4%)** were investigated, **154 suspected cases including 40 deaths** were identified (Table 3).

Table 4. Contact tracing of confirmed cases as of June 24, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri		5,545	78.1%
North Kivu	7,103,2184	1805	82.6%
South Kivu	18	18	100.0%
Total	9,305	7,368	79.2%

- Continued in-depth investigations and exhaustive listing and systematic follow-up of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and followed per case).
- 311 new contacts** were listed in 9 health zones in Ituri province. On the other hand, **99 contacts have successfully completed their 21-day monitoring, including 79 days in North Kivu and 20 in Ituri.**

Update on checkpoint and point of entry (PoC/PoE) activities

- In Ituri :** three (3) alerts were notified to the PoE/PoC, including one live alert that was validated and referred to the Aru CTE from the Apinaka/ZS Aru PoC; two alerts were invalidated after secondary screening or observation at the FONER Mambasa and OFOO PoC/PoEs. No bodies were intercepted today.

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of June 24, 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	85.5% (47/55)	92.3% (12/13)	86.8% (59/68)
Number of people who went through PoE/PoC testing,	92,533	171,591	264,124
% of travelers screened, % of	96.2%	97.0%	96.7%
travelers who washed their hands, % of	92.4%	97.5%	95.7%
travelers made aware of the issue	94.7%	97.0%	89.3%
Number of alerts notified	3	0	3
Number of validated live alerts	1	0	1
Number of lifeless bodies intercepted today	0	0	0
Number of problematic contacts intercepted today; %	0	0	0
of validated active alerts referred to CT/CTE	100.0%	0.0%	100.0%
% of the lifeless bodies swabbed	100.0%	0.0%	100.0%
Number of positive results of referred suspected cases	0	0	0

4.3. Laboratory

- Ituri :** 134 samples collected and analyzed (positivity 25.0%; n=34).
- North Kivu :** Out of a total of 117 samples received, 97 were analyzed, **3 came back positive (3.1%)** and 20 are still being analyzed.
- South Kivu :** 6 samples were collected and analyzed, all came back negative.

4.4. Infection Prevention and Control (IPC)

Ituri: 22/27 safe and dignified burials (SDBs) were carried out, with 5 postponements. IPC/WASH activities included 10 Scorecard assessments in health centers, 20 supported health centers with 81

Providers briefed on standard precautions; 17 households, 4 social and solidarity economy establishments and 16 vehicles decontaminated; 55 community leaders briefed on Ebola virus disease and standard precautions.

North Kivu : Joint WHO and PCI WASH Pillar mission to the Oicha Health Zone including a visit to the Oicha Technical Center with identification of gaps and provision of PCI WASH inputs to the Oicha Health Zone; joint UNICEF and PCI WASH Pillar mission to the Carl Beker Health Center for monitoring the operation of the photovoltaic borehole and sorting unit; monitoring the operation of the PLM installed at the entrance of the Beni central market; water supply to the Katwa Water Treatment Center cisterns by CAUB, implementing partner of the UNICEF funding, to cover water needs.

South Kivu : Closing of the port and undressing simulation workshop, EDS, coaching on the use of the Kobocollect tool for PCI score card assessments to the 45 members of the PCI pillar in RIO.

4.5 Holistic care

- **Sixteen (16) patients** were declared cured in Ituri, after obtaining negative control results.

Thus, the cumulative number of recovered patients is **138, including 129 in Ituri, 7 in North Kivu and 2 in South Kivu.**

- Continued medical care, transfers to CTE/CT facilities, announcements of results, and nutritional monitoring of patients. In total, 660 hot meals were served, including 111 to confirmed cases, 100 to suspected cases, 387 to PPL (People with Disabilities) and 62 to accompanying persons.

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of June 24, 2026

Indicator	Ituri	North Kivu*	South Kivu	Together
Patients in bed (Day -1)	322	56	12	390
Total admissions (24h)	59	8	6	73
Exits (24h)				
Deceased (Suspects/Confessions)	14	0	0	14
No case	37	5	5	47
Healed	16	0	0	16
Escapees (Suspects/Confessions)	1	0	0	1
Transferred to the HGR	0	0	0	0
Total outflows	68	5	5	78
Patients in isolation (End J)	313	59	13	385
including confirmed	188	17	0	205
(NC+AC)	125	42	13	180
including suspected	85.3%	57.3%	29.5%	74.9%

Overall occupancy rate *Data reported for North Kivu province are from June 23, 2026 because data from June 24, 2026 were not available at the time of consolidation of this report.

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of June 24, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	6 (30.0%)	1 (100.0%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	49 (55.7%)	7 (100.0%)	0
New suspected cases that benefited from SMSPS interventions	7 (22.6%)	8 (100.0%)	1 (100.0%)
Suspected cases under follow-up that have received interventions SMSPS	45 (52.9%)	18 (100.0%)	3 (100.0%)
Laboratory results have been announced.	17	15	0
Including positive ones	6	1	0
Number of children separated in daycare who benefited from interventions SMSPS	0 (0.0%)	0 (0.0%)	0 (0.0%)
Number of children separated in the community who benefited from SMSPS interventions	0	0	23
Number of accompanying persons at the CTE who benefited from interventions SMSPS	64	13	3
Number of PPLs who benefited from SMSPS interventions	0	8	17
Number of household and community members who benefited from SMSPS interventions	53	29	0
Number of EDS supported	0	1	0

4.6 Continuity of care

- Monitoring of CTE extension work, particularly Rwampara and HGR/Bunia; progressive improvement of the technical platform; transfer of patients from ESS/communities to the sites selected for care; briefing on data reporting to the Elikya CTE.

4.7 Risk communication and community engagement

Ituri: CREC activities focused on verifying the existence of active RECOs (Regional Community Education Networks) in the urban-rural area of Bunia, broadcasting an interactive program featuring a testimonial from a survivor of the Rwampara Ebola Treatment Center (ETC) via 17 partner radio stations, and conducting awareness campaigns in mining and community areas. In total, 3 community dialogues and 19 mass awareness sessions reached 1,886 people; 394 households were visited, and 259 educational talks reached 4,521 people. Eighteen (18) public service announcements/messages on preventive measures for viral shedding were broadcast with support from partner FHI 360.

Support for the pillars of the response : Surveillance: investigation and contact tracing, particularly in Mongbwalu and Nyankunde; it also facilitated the return of 5 suspected escapees from AS Saio to the Mongbwalu General Referral Hospital's treatment center. **For infection prevention and control (IPC),** it contributed to household adherence to decontamination and acceptance of emergency health measures (EHS).

North Kivu: In the Manguredjipa Health Zone: Public forum with 491 motorcycle taxi drivers on commitment to risk reduction through adherence to Ebola prevention measures. In the Katwa and Butembo Health Zones: Meeting with CREC Supervisors on strengthening support and

Support for the Butembo and Katwa health zones. In the Vuhovi health zone : briefing of traditional healers and managers of private structures on risk communication, community-based surveillance and community engagement in the fight against Ebola virus disease.

South Kivu : 1,059 households visited, 9,510 people sensitized, 54 leaders engaged; 10 radio stations broadcast spots and programs on Ebola Virus Disease (EVD); briefing and advocacy with 54 community leaders, religious leaders, youth and women representatives from the Kahungu, Lwiro, Chaoboka and Buhandahanda health districts in the Miti-murhesa health zone on EVD and their contribution to facilitating public health actions (decontamination, blood and sanitation services, sampling, contact tracing); briefing of 6 IOM service providers at the Lwiro handwashing site in the Miti-murhesa health zone on communication techniques with passengers; educational talks with 19 butchers in the Katana center on prevention measures and signs of EVD; mass awareness campaign with 477 final-year students in the Katana health zone on prevention measures and signs of EVD.

4.8. Logistics

Ituri

- In total, 23 vehicles and 4 ambulances were made available to the various pillars to ensure their mobility; ten (10) vehicle contracts were renewed with the support of the UG-PDSS.
- Supply of inputs for the Wellbeing and Surveillance pillars from the DPS stock; supplementary PCI inputs for the Kilo Health Zone; preparation of medicine kits for the Nia-Nia Health Zone; delivery of 12 packages of Tyvek (98 kg) for the Mongbwalu Health Zone and 22 packages of PCI inputs (202 kg) for training in the Nizi Health Zone.
- Infrastructure management: continuation of work on the new CTE at HGR/Bunia, commissioning of 12 new tents at CME/Bunia and installation of a tent for the Well-being pillar.

North Kivu

- Delivery of UG-PDSS medicines to the health zones of Butembo, Kalunguta and Katwa; Delivery PCI inputs to the Beni Health Zone
- Deployment of teams from the different pillars in the health zones; Delivery of PCI inputs to the Poc and PoE sub-pillar for the Kasitu PoC.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- Continued briefings for stakeholders, community awareness-raising on exploitation, abuse and sexual harassment, as well as the signing of codes of good conduct.
- **South Kivu :** A briefing and advocacy session was held with 54 (17 women) community leaders, religious leaders and village chiefs from the health areas of Lwiro, Kahungu, Chaoboka and Buhandahanda, in the Miti-Murhesa Health Zone, on the principle of zero tolerance towards exploitation and abuse, the green line 495555. They committed to promoting responsible behavior, preventing all forms of abuse in order to strengthen community trust in the response teams.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity in CTE and close to saturation in Ituri (85.3%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3 Poor performance in contact tracing (79.2% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4. Low reporting of alerts in certain affected zones	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not localized or unclassified (17 cases)	Major gaps in the reconstruction of transmission chains	Conducting a data reconciliation, Remove any potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (78 cases) HCW (including 18 deaths, case fatality rate 23.1%)	Urgent supply, strengthening of IPC training
8 Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (54.1%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities
9 Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10. Insufficient coordination and low utilization of COUSP frameworks	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, make operational the Provincial COUSP
11. Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12 Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13 Continuity of essential services (CEHS) compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14. High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

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